

Ageas Federal Life Insurance **Criti Care Plan**

A Non-Linked, Non-Participating, Individual, Health Insurance Plan
(UIN: 135N085V01)

Stay Insured

Stay Fearless



Introduction

We aspire to lead a happy and content life. An essential aspect of happiness is good health. So while a critical illness or major disease may strike unexpectedly, it is prudent to be covered with a Critical Illness Plan- that provides financial support to cover expensive treatment costs, medical bills and even during the recuperation period.

Introducing Ageas Federal Life Insurance Criti Care Plan, that covers 30 critical illnesses and provides Sum Assured payment on diagnosis of any of the covered critical illnesses. Here's a plan that understands the importance of your hard-earned savings and safeguards financially by easing the burden of expensive medical care and possible loss of income during the recovery period.

Plan at a Glance

Ageas Federal Life Insurance Criti Care Plan has been tailored keeping your needs in mind and comes with a host of features:

- 30 Critical Illnesses covered
- Flexibility of Policy Term and Premium Paying Term options.
- Level Critical Illness cover throughout the Policy Term.
- Offers lump-sum pay-out on the first diagnosis of illness irrespective of actual amount spent on treatment.
- Tax Benefit may be available on the premium paid under Section 80 D*

*Please note that tax benefits are subject to change as per tax laws. Consult your tax advisor for determining the Tax benefits applicable to you.

Plan in Detail

Comprehensive Coverage

The plan provides assured Critical Illness benefit on being diagnosed after waiting period with any of the listed 30 critical illnesses during the policy term, provided the policy is in force with all premium due being paid to date, Sum Assured shall be payable in lumpsum.

On payment of such benefit, the policy shall terminate and all rights, benefits and interests under the cover shall stand extinguished.

List of 30 Critical illnesses covered

Cancer of Specified Severity
Myocardial Infarction (First Heart Attack of specific severity)
Open Chest CABG
Open Heart Replacement or Repair of Heart Valves
Coma of Specified Severity
Kidney Failure Requiring Regular Dialysis
Stroke Resulting in Permanent Symptoms
Major Organ/ Bone Marrow Transplant
Permanent Paralysis of Limbs
Motor Neuron Disease with Permanent Symptoms
Multiple Sclerosis with Persisting Symptoms
Benign Brain Tumor
Blindness
Deafness
End Stage Lung Failure

End Stage Liver Failure
Loss of Speech
Loss of Limbs
Major Head Trauma
Primary (Idiopathic) Pulmonary Hypertension
Third Degree Burns
Alzheimer's Disease
Apallic Syndrome
Aplastic Anaemia
Loss of Independent Existence
Medullary Cystic Disease
Muscular Dystrophy
Parkinson's Disease
Poliomyelitis
Aorta Graft Surgery



Tax benefits under Sec 80D of Income Tax Act, 1961

Tax benefit may be available on the premiums paid as per prevailing tax laws.

Deduction under section 80D: The premiums that you pay may be eligible for deduction under section 80D of the Income Tax Act, 1961.



Death Benefit

The policy does not provide any Death Benefit



Maturity Benefit

The policy does not provide any Maturity Benefit.



Paid-up Benefit

The policy does not provide any Paid-up benefit.



Surrender Value

This is a pure protection plan and therefore does not acquire surrender value for any premium payment options i.e., Single Pay, Limited Pay and Regular Pay.

However, following Refund Value (as applicable) shall be payable.

● For Single Pay

The policyholder voluntarily opts to close the policy anytime during the policy term.

Refund Value is:

$60\% \times \text{Single Premium Paid} \times (\text{Unexpired Policy Term in months} / \text{Total Policy Term in months});$.

“Single Premium“ means premium received, excluding any extra premium and taxes.

● For Limited Pay

The plan acquires Refund Value if all premiums have been paid in full for at least the first three consecutive years and payable upon earlier occurrence of:

- Critical Illness claim during revival period where policy has lapsed
- End of revival period if policy is not reinstated.
- Policyholder voluntarily exiting the policy.

Refund Value is:

$60\% \times \text{Total Premiums Paid till date} \times \{(\text{Policy Term} - \text{Premium Payment Term}) / \text{Policy Term}\} \times (\text{Unexpired Policy Term in months} / \text{Total Policy Term in months});$

“Total Premiums Paid” means total of all the premiums received, excluding any extra premium, any rider premium and taxes.

● For Regular Pay

The plan does not acquire Refund Value under regular premium paying policies.

On payment of Refund Value, the policy shall terminate and all rights, benefits and interests under the policy stands extinguished.

Loans

No loans are available under this plan.

Eligibility

Age at entry of Proposer (as on last birthday)	Min	18 years
	Max	99 years
Age at entry of Life Assured (as on last birthday)	Min	18 years
	Max	65 years
Age at maturity of Life Assured (as on last birthday)	Min	23 years
	Max	70 years
Sum Assured*	Min	Rs. 2,00,000
	Max	Rs. 50,00,000
Premium Paying Frequency	Yearly	
Premium Paying Term	Policy Term	
Single Pay	5 to 10 years	
Regular Pay	5 to 10 Years	
Limited Pay 5 and 7 Years	10 years	
Annualized Premium	Min	Rs. 468 (Age: 18 years, PT: 5 years Regular Pay, Female, Sum Assured: Rs. 2,00,000, Staff Discount: Yes)
	Max	No limit, subject to Board Approved Underwriting Policy.

^All ages are as per age last birthday.

*Sum Assured for standard lives shall be in multiples of Rs. 1,00,000. For sub-standard life, Sum Assured at inception shall be as per Board Approved Underwriting Policy

Illustration

Rakesh is a 35 year old project manager in an IT company. Realising the need to cover Critical Illness in the future and be well prepared for unforeseen circumstances in life, he opts for Ageas Federal Life Insurance Citi Care Plan. On choosing the policy term of 5 years and Sum Assured of Rs. 15 Lakh with regular premium pay option, he pays an annual premium of Rs. 4,245. At age 38, Rakesh is diagnosed with Critical Illness. On such diagnosis he receives the Sum Assured of Rs. 15 Lakh as a lump sum. On payment of the Sum Assured, the policy shall terminate and all rights, benefits and interests under the policy shall stand extinguished.

Sample Premium Rates

Please find below the table containing sample premium rates for healthy male with a policy term of 5years, regular premium pay option and a sum assured of Rs.15 Lakhs

Age	Premium Amount in Rs.
25	2,100
30	2,775
35	4,245
40	8,175
45	15,855
50	23,235

**The above mentioned premiums are exclusive of loadings if any & GST.*

Other Terms and Conditions



Grace Period

Policyholder is required to pay his renewal premium every year on or before the scheduled premium payment due dates. For such payment, policyholder gets the facility of a grace period to pay the due premium. The grace period is of 30 days effective from the date of the first unpaid premium. The benefits of the policy remain in force during the grace period. If premium is not paid during the grace period, then all benefits under the policy will cease immediately and the policy will be deemed lapsed.

In case of any claim during grace period before, the premium due at that time is paid, benefit payable would be paid after deducting the premiums as due up to the next policy anniversary, without any interest.



Policy Lapse

- **For Regular and Limited Pay:**

In case of non-payment of due Premiums within the grace period, the policy shall lapse and no benefits are payable except refund value.

However, policy can be reinstated subject to the conditions as mentioned under Section policy revival.

- **For Single Pay:**

This is not applicable for Single Pay policies.

Policy Revival

A policy which has lapsed may be revived subject to the following conditions:

A policy which has lapsed can be revived subject to the following conditions:

- An application for revival is made within 5 consecutive years from the due date of the first unpaid premium or before the end of policy term, whichever is earlier.
- The Life Assured has furnished satisfactory evidence of health and other requirements as per the Company's board approved underwriting guidelines at that time. Medical tests, if required, have to be borne by the policyholder at his own cost.
- The arrears of premium together with interest, at such rate as decided by the Company from time to time, are paid along with the revival application.

The interest on revival, if any will be decided by the company from time to time. The interest if so decided shall be set as quarterly equivalent of (3% + annualized yield on 10 year Government security). Annualized Yield on 10 year Government security is sourced through Financial Benchmarks India Pvt. Ltd. (FBIL). Any change in this formula and basis to set interest rates shall be made with necessary approvals as required by IRDAI. (The current rate of interest applicable from 1st August, 2022 is 10.60% per annum basis 10 years G-sec rate as on 30th June, 2022. Interest rate shall be compounded annually. The interest rate will be reviewed by the Company board, every 6 months. The same will be reset every year on 1st February and 1st August. Interest charged on policy revival is compounded annually. Such

revival shall be based on the Board approved underwriting policy.

Once a policy has been revived, all benefits will be reinstated as per the terms and conditions of the policy.



Premium Guarantee

The premium rates are guaranteed for the entire policy term.



Waiting Period

A waiting period of 90 days from the risk commencement date or the date of revival, whichever is later, is applicable for admissibility of a claim. There will be no waiting period applicable for Critical Illness claim arising solely due to an accident.

If the Life Assured is diagnosed to have any one of the covered critical illnesses during the waiting period, provided the policy is in force with all premiums due being paid to date, 100% of the Total Premiums Paid* till date of claim shall only be refunded.

*Total Premiums Paid means total of all the premiums received, excluding any extra premium, any rider premium and taxes.

On payment of such benefit, the policy shall terminate and all rights, benefits and interests under the policy shall stand extinguished.



Survival Period

Survival Period of 30 days will be applicable for admissibility of a claim on listed Critical illnesses. Survival Period refers to the period from the diagnosis and fulfilment of the definition of the conditions covered for which the Life Assured must survive before for admissibility of a claim. The benefit shall be payable only on confirmatory diagnosis of the conditions covered while the Life Assured is alive.



Assignment & Nomination

Assignment will be allowed in accordance with provisions of Section 38 of the Insurance Act, 1938 as amended from time to time.

Nomination will be allowed in accordance with provisions of Section 39 of the Insurance Act, 1938 as amended from time to time.



Goods & Services Tax

Goods and Services Tax, and cess as applicable, will be levied as per the extant laws. The current GST applicable under the policy is 18% of the premiums paid from the first policy year onwards.



Free-look Period

You are entitled to a free look period of 30 days beginning from the date of receipt of the policy document (whether received electronically or otherwise), to review the terms and conditions of the policy. In case you do not agree to any of the policy terms and conditions, or otherwise and have not made any claim, you have the option to return the policy to us for cancellation by communicating the same in writing stating the reasons for cancellation. We will refund you the premium paid after deducting the

proportionate risk premium for the period of cover provided and the expenses, if any, incurred by us in respect of medical examination and stamp duty charges. All the benefits under the policy will stand extinguished immediately on the cancellation of the Policy under the free look.



Renewal Premium in Advance

Collection of renewal premium in advance shall be allowed within the same financial year for the premium due in that financial year. Provided, the premium due in one financial year may be collected in advance in earlier financial year for a maximum period of three months in advance of the due date of the premium.

The renewal premium so collected in advance shall only be adjusted on the due date of the premium.

DEFINITIONS OF COVERED CRITICAL ILLNESSES

1. Cancer of Specified Severity

A malignant tumor characterized by the uncontrolled growth & spread of malignant cells with invasion & destruction of normal tissues. This diagnosis must be supported by histological evidence of malignancy. The term cancer includes leukemia, lymphoma and sarcoma.

The following are excluded –

- i. All tumors which are histologically described as carcinoma in situ, benign, pre-malignant, borderline malignant, low malignant potential, neoplasm of unknown behavior, or non-invasive, including but not limited to: Carcinoma in situ of breasts, Cervical dysplasia CIN-1, CIN -2 and CIN-3.
- ii. Any non-melanoma skin carcinoma unless there is evidence of metastases to lymph nodes or beyond;
- iii. Malignant melanoma that has not caused invasion beyond the epidermis;
- iv. All tumors of the prostate unless histologically classified as having a Gleason score greater than 6 or having progressed to at least clinical TNM classification T2N0M0
- v. All Thyroid cancers histologically classified as T1N0M0 (TNM Classification) or below;
- vi. Chronic lymphocytic leukaemia less than RAI stage 3
- vii. Non-invasive papillary cancer of the bladder histologically described as TaN0M0 or of a lesser classification,
- viii. All Gastro-Intestinal Stromal Tumors histologically classified as T1N0M0 (TNM Classification) or below and with mitotic count of less than or equal to 5/50 HPFs;

2. Myocardial Infarction (First Heart Attack of Specific Severity)

I.) The first occurrence of heart attack or myocardial infarction, which means the Death of a portion of the heart muscle as a result of inadequate blood supply to the relevant area. The diagnosis for Myocardial Infarction should be evidenced by all of the following criteria:

- i. A history of typical clinical symptoms consistent with the diagnosis of Acute Myocardial Infarction (for e.g. typical chest pain)
- ii. New characteristic electrocardiogram changes
- iii. Elevation of infarction specific enzymes, Troponins or other specific biochemical markers.

II.) The following are excluded:

- i. Other acute Coronary Syndromes
- ii. Any type of angina pectoris
- iii. A rise in cardiac biomarkers or Troponin T or I in absence of overt ischemic heart disease OR following an intra-arterial cardiac procedure

3. Open Chest CABG

I.) The actual undergoing of heart surgery to correct blockage or narrowing in one or more coronary artery(s), by coronary artery bypass grafting done via a sternotomy (cutting through the breastbone) or minimally invasive keyhole coronary artery bypass procedures. The diagnosis must be supported by a coronary angiography and the realization of surgery has to be confirmed by a cardiologist

II.) The following are excluded:

- i. Angioplasty and/or any other intra-arterial procedures

4. Open Heart Replacement or Repair of Heart Valves

The actual undergoing of open-heart valve surgery to replace or repair one or more heart valves, as a consequence of defects in, abnormalities of, or disease-affected cardiac valve(s). The diagnosis of the valve abnormality must be supported by an echocardiography and the realization of surgery has to be confirmed by a specialist medical practitioner. Catheter based techniques including but not limited to, balloon valvotomy/valvuloplasty are excluded.

5. Coma of specified Severity

I.) A state of unconsciousness with no reaction or response to external stimuli or internal needs. This diagnosis must be supported by evidence of all of the following:

- i. No response to external stimuli continuously for at least 96 hours;
- ii. Life support measures are necessary to sustain life; and
- iii. Permanent neurological deficit which must be assessed at least 30 days after the onset of the coma.

II.) The condition has to be confirmed by a specialist medical practitioner. Coma resulting directly from alcohol or drug abuse is excluded.

6. Kidney Failure Requiring Regular Dialysis

End stage renal disease presenting as chronic irreversible failure of both kidneys to function, as a result of which either regular renal dialysis (haemodialysis or peritoneal dialysis) is instituted or renal transplantation is carried out. Diagnosis has to be confirmed by a specialist medical practitioner.

7. Stroke resulting in permanent symptoms

I.) Any cerebrovascular incident producing permanent neurological sequelae. This includes infarction of brain tissue, thrombosis in an intracranial vessel, haemorrhage and embolisation from an extracranial source. Diagnosis has to be confirmed by a specialist medical practitioner and evidenced by typical clinical symptoms as well as typical findings in CT Scan or MRI of the brain. Evidence of permanent neurological deficit lasting for at least 3 months has to be produced.

II.) The following are excluded:

- i. Transient ischemic attacks (TIA)
- ii. Traumatic injury of the brain
- iii. Vascular disease affecting only the eye or optic nerve or vestibular functions.

8. Major Organ/ Bone Marrow Transplant

I.) The actual undergoing of a transplant of:

- i. One of the following human organs: heart, lung, liver, kidney, pancreas, that resulted from irreversible end-stage failure of the relevant organ, or
- ii. Human bone marrow using haematopoietic stem cells. The undergoing of a transplant has to be confirmed by a specialist medical practitioner.

II.) The following are excluded:

- i. Other stem-cell transplants
- ii. Where only Islets of Langerhans are transplanted

9. Permanent Paralysis of Limbs

I.) Total and irreversible loss of use of two or more limbs as a result of injury or disease of the brain or spinal cord. A specialist medical practitioner must be of the opinion that the paralysis will be permanent with no hope of recovery and must be present for more than 3 months.

10. Motor Neuron Disease with Permanent Symptoms

I.) Motor neuron disease diagnosed by a Specialist Medical Practitioner as spinal muscular atrophy, progressive bulbar palsy, amyotrophic lateral sclerosis or primary lateral sclerosis. There must be progressive degeneration of corticospinal tracts and anterior horn cells or bulbar efferent neurons. There must be current significant and permanent functional neurological impairment with objective evidence of motor dysfunction that has persisted for a continuous period of at least 3 months.

11. Multiple Sclerosis with Persisting Symptoms

- I.) The unequivocal diagnosis of Definite Multiple Sclerosis confirmed and evidenced by all of the following:
- i. investigations including typical MRI findings which unequivocally confirm the diagnosis to be multiple sclerosis and
 - ii. there must be current clinical impairment of motor or sensory function, which must have persisted for a continuous period of at least 6 months.
- II.) Neurological damage due to SLE is excluded.

12. Benign Brain Tumor

- I.) Benign brain tumor is defined as a life threatening, non-cancerous tumor in the brain, cranial nerves or meninges within the skull. The presence of the underlying tumor must be confirmed by imaging studies such as CT scan or MRI.
- II.) This brain tumor must result in at least one of the following and must be confirmed by the relevant medical specialist:
- i. Permanent Neurological deficit with persisting clinical symptoms for a continuous period of at least 90 consecutive days or
 - ii. Undergone surgical resection or radiation therapy to treat the brain tumor.
- III.) The following conditions are excluded:
- Cysts, Granulomas, malformations in the arteries or veins of the brain, hematomas, abscesses, pituitary tumors, tumors of skull bones and tumors of the spinal cord.

13. Blindness

Total, permanent and irreversible loss of all vision in both eyes as a result of illness or accident.

The Blindness is evidenced by:

- i. corrected visual acuity being 3/60 or less in both eyes or;
- ii. the field of vision being less than 10 degrees in both eyes.

The diagnosis of blindness must be confirmed and must not be correctable by aids or surgical procedure.

14. Deafness

Total and irreversible loss of hearing in both ears as a result of illness or accident. This diagnosis must be supported by pure tone audiogram test and certified by an Ear, Nose and Throat (ENT) specialist. Total means “the loss of hearing to the extent that the loss is greater than 90decibels across all frequencies of hearing” in both ears.

15. End Stage Lung Failure

I.) End stage lung disease, causing chronic respiratory failure, as confirmed and evidenced by all of the following:

- i. FEV1 test results consistently less than 1 litre measured on 3 occasions 3 months apart; and
- ii. Requiring continuous permanent supplementary oxygen therapy for hypoxemia; and
- iii. Arterial blood gas analysis with partial oxygen pressures of 55mmHg or less ($\text{PaO}_2 < 55 \text{ mmHg}$); and
- iv. Dyspnea at rest.

16. End Stage Liver Failure

I.) Permanent and irreversible failure of liver function that has resulted in all three of the following:

- i. permanent jaundice; and
- ii. ascites; and
- iii. hepatic encephalopathy.

II.) Liver failure secondary to drug or alcohol abuse is excluded.

17. Loss of speech

I.) Total and irrecoverable loss of the ability to speak as a result of injury or disease to the vocal cords. The inability to speak must be established for a continuous period of 12 months. This diagnosis must be supported by medical evidence furnished by an Ear, Nose, Throat (ENT) specialist.

18. Loss of Limbs

I.) The physical separation of two or more limbs, at or above the wrist or ankle level limbs as a result of injury or disease. This will include medically necessary amputation necessitated by injury or disease. The separation has to be permanent without any chance of surgical correction. Loss of Limbs resulting directly or indirectly from self-inflicted injury, alcohol or drug abuse is excluded.

19. Major Head Trauma

I.) Accidental head injury resulting in permanent Neurological deficit to be assessed no sooner than 3 months from the date of the accident. This diagnosis must be supported by unequivocal findings on Magnetic Resonance Imaging, Computerized Tomography, or other reliable imaging techniques. The accident must be caused solely and directly by accidental, violent, external and visible means and independently of all other causes.

II.) The Accidental Head injury must result in an inability to perform at least three (3) of the following Activities of Daily Living either with or without the use of mechanical equipment, special devices or other aids and adaptations in use for disabled persons. For the purpose of this benefit, the word “permanent” shall mean beyond the scope of recovery with current medical knowledge and technology.

III.) The Activities of Daily Living are:

- i. Washing: the ability to wash in the bath or shower (including getting into and out of the bath or shower) or wash satisfactorily by other means.
- ii. Dressing: the ability to put on, take off, secure, and unfasten all garments and, as appropriate, any braces, artificial limbs or other surgical appliances.
- iii. Transferring: the ability to move from a bed to an upright chair or wheelchair and vice versa.
- iv. Mobility: the ability to move indoors from room to room on level surfaces.
- v. Toileting: the ability to use the lavatory or otherwise manage bowel and bladder functions so as to maintain a satisfactory level of personal hygiene.
- vi. Feeding: the ability to feed oneself once food has been prepared and made available.

IV.) The following is excluded:

- i. Spinal cord injury.

20. Primary (Idiopathic) Pulmonary Hypertension

I.) An unequivocal diagnosis of Primary (Idiopathic) Pulmonary Hypertension by a Cardiologist or specialist in respiratory medicine with evidence of right ventricular enlargement and the pulmonary artery pressure above 30 mm of Hg on Cardiac Catheterization. There must be permanent irreversible physical impairment to the degree of at least Class IV of the New York Heart Association Classification of cardiac impairment.

II.) The NYHA Classification of Cardiac Impairment are as follows:

- i. Class III: Marked limitation of physical activity. Comfortable at rest, but less than ordinary activity causes symptoms.
- ii. Class IV: Unable to engage in any physical activity without discomfort. Symptoms may be present even at rest.

III.) Pulmonary hypertension associated with lung disease, chronic hypoventilation, pulmonary thromboembolic disease, drugs and toxins, diseases of the left side of the heart, congenital heart disease and any secondary cause are specifically excluded.

21. Third Degree Burns

There must be third-degree burns with scarring that cover at least 20% of the body's surface area. The diagnosis must confirm the total area involved using standardized, clinically accepted, body surface area charts covering 20% of the body surface area.

22. Alzheimer's Disease

Alzheimer's (presenile dementia) disease is a progressive degenerative disease of the brain, characterised by diffuse atrophy throughout the cerebral cortex with distinctive histopathological changes. It affects the brain, causing symptoms like memory loss, confusion, communication problems, and general impairment of mental function, which gradually worsens leading to changes in personality. Deterioration or loss of intellectual capacity, as confirmed by clinical evaluation and imaging tests, arising from Alzheimer's disease, resulting in progressive significant reduction in mental and social functioning, requiring the continuous supervision of the Life Assured. The diagnosis must be supported by the clinical confirmation of a Neurologist and supported by Our appointed Medical Practitioner. The disease must result in a permanent inability to perform three or more Activities of daily living with Loss of Independent Living" or must require the need of supervision and permanent presence of care staff due to the disease. This must be medically documented for a period of at least 90 days.

The following conditions are however not covered :

- a) neurosis or neuropsychiatric symptoms without imaging evidence of Alzheimer's Disease.
- b) alcohol related brain damage; and
- c) any other type of irreversible organic disorder/dementia not associated with Alzheimer's Disease

Activities of Daily Living:

- i. Washing: the ability to wash in the bath or shower (including getting into and out of the shower) or wash satisfactorily by other means.
- ii. Dressing: the ability to put on, take off, secure and unfasten all garments and, as appropriate, any braces, artificial limbs or other surgical appliances.
- iii. Transferring: the ability to move from a bed to an upright chair or wheelchair and vice versa.

- iv. Toileting: the ability to use the lavatory or otherwise manage bowel and bladder functions so as to maintain a satisfactory level of personal hygiene.
- v. Feeding: the ability to feed oneself once food has been prepared and made available.
- vi. Mobility: the ability to move indoors from room to room on level surfaces;

23. Apallic Syndrome

Apallic Syndrome or Persistent vegetative state (PVS) or unresponsive wakefulness syndrome (UWS) is a Universal necrosis of the brain cortex with the brainstem remaining intact. The diagnosis must be confirmed by a Neurologist acceptable to us and the patient should be documented to be in a vegetative state for a minimum of at least one month in order to be classified as UWS, PVS, Apallic Syndrome.

24. Aplastic Anaemia

Chronic persistent bone marrow failure which results in anaemia, neutropenia and thrombocytopenia requiring treatment with at least one of the following:

- a. Blood product transfusion.
- b. Marrow stimulating agents.
- c. Immunosuppressive agents or
- d. Bone marrow transplantation.

The diagnosis must be confirmed by a haematologist using relevant laboratory investigations including Bone Marrow Biopsy resulting in bone marrow cellularity of less than 25% which is evidenced by any two of the following:

- a. Absolute neutrophil count of less than $500/\text{mm}^3$ or less
- b. Platelets count less than $20,000/\text{mm}^3$ or less
- c. Reticulocyte count of less than $20,000/\text{mm}^3$ or less Temporary or reversible Aplastic Anaemia is excluded.

25. Loss of Independent Existence (cover up to Insurance Age 70)

The Life Assured person is physically incapable of performing at least three (3) of the “Activities of Daily Living” as defined below (either with or without the use of mechanical equipment, special devices or other aids or adaptations in use for disabled persons) for a continuous period of at least six (6) months, signifying a permanent and irreversible inability to perform the same. For the purpose of this definition, the word “permanent” shall mean beyond the hope of recovery with

current medical knowledge and technology. The Diagnosis of Loss of Independent Existence must be confirmed by a Registered Doctor who is a specialist. Only Life Assured with Insurance Age between 18 and 70 on first diagnosis is eligible to receive a benefit under this illness.

Activities of daily living are:

- i. Washing: the ability to wash in the bath or shower (including getting into and out of the shower) or wash satisfactorily by other means.
- ii. Dressing: the ability to put on, take off, secure and unfasten all garments and, as appropriate, any braces, artificial limbs or other surgical appliances.
- iii. Transferring: the ability to move from a bed to an upright chair or wheelchair and vice versa.
- iv. Toileting: the ability to use the lavatory or otherwise manage bowel and bladder functions so as to maintain a satisfactory level of personal hygiene.
- v. Feeding: the ability to feed oneself once food has been prepared and made available.
- vi. Mobility: The ability to move indoors from room to room on level surfaces.

26. Medullary Cystic Disease

Medullary Cystic Disease where the following criteria are met:

- a) the presence in the kidney of multiple cysts in the renal medulla accompanied by the presence of tubular atrophy and interstitial fibrosis;
- b) clinical manifestations of anaemia, polyuria, and progressive deterioration in kidney function; and
- c) the Diagnosis of Medullary Cystic Disease is confirmed by renal biopsy. Isolated or benign kidney cysts are specifically excluded from this benefit.

27. Muscular Dystrophy

A group of hereditary degenerative diseases of muscle characterised by weakness and atrophy of muscle. The diagnosis of muscular dystrophy must be unequivocal and made by a Registered Doctor who is a consultant neurologist. The condition must result in the inability of the Life Assured to perform (whether aided or unaided) at least 3 of the 6 “Activities of Daily Living” for a continuous period of at least 6 months.

Activities of daily living:

- i. Washing: the ability to wash in the bath or shower (including getting into and out of the shower) or wash satisfactorily by other means;

- ii. Dressing: the ability to put on, take off, secure and unfasten all garments and, as appropriate, any braces, artificial limbs or other surgical appliances;
- iii. Transferring: the ability to move from a bed to an upright chair or wheelchair and vice versa;
- iv. Toileting: the ability to use the lavatory or otherwise manage bowel and bladder functions so as to maintain a satisfactory level of personal hygiene;
- v. Feeding: the ability to feed oneself once food has been prepared and made available;
- vi. Mobility: The ability to move indoors from room to room on level surfaces.

28. Parkinson's disease

The unequivocal diagnosis of progressive, degenerative idiopathic Parkinson's disease by a Neurologist acceptable to us.

The diagnosis must be supported by all of the following conditions:

- a. the disease cannot be controlled with medication;
- b. signs of progressive impairment; and
- c. inability of the Life Assured to perform at least 3 of the 6 activities of daily living as listed below (either with or without the use of mechanical equipment, special devices or other aids and adaptations in use for disabled persons) for a continuous period of at least 6 months:

Activities of daily living:

- i. Washing: the ability to wash in the bath or shower (including getting into and out of the shower) or wash satisfactorily by other means;
- ii. Dressing: the ability to put on, take off, secure and unfasten all garments and, as appropriate, any braces, artificial limbs or other surgical appliances;
- iii. Transferring: The ability to mover from bed to a upright chair or wheelchair and vice versa;
- iv. Toileting: the ability to use the lavatory or otherwise manage bowel and bladder functions so as to maintain a satisfactory level of personal hygiene;
- v. Feeding: The ability to feed oneself once the food has prepared and made available;
- vi. Mobility: The ability to move indoors from room to room on level surfaces. Parkinson's disease secondary to drug and/or alcohol abuse is excluded.

29. Poliomyelitis

The occurrence of Poliomyelitis where the following conditions are met:

- i. Poliovirus is identified as the cause,
- ii. Paralysis of the limb muscles or respiratory muscles must be present and persist for at least 3 months.

30. Aorta Graft Surgery

The actual undergoing of major Surgery to repair or correct aneurysm, narrowing, obstruction or dissection of the Aorta through surgical opening of the chest or abdomen. For the purpose of this cover the definition of “**Aorta**” shall mean the thoracic and abdominal aorta but not its branches.

The Life Assured understands and agrees that we will not cover:

- a. Surgery performed using only minimally invasive or intra-arterial techniques.
- b. Angioplasty and all other intra-arterial, catheter based techniques, “keyhole” or laser procedures. Aorta graft surgery benefit covers Surgery to the aorta wherein part of it is removed and replaced with a graft.

Permanent Exclusions for Critical Illness:

In addition to any condition specific exclusions mentioned in the definitions above, We shall not be liable to pay any claim arising out of, due to any of the following:

1. Any Illness, sickness or disease other than those specified as Critical Illnesses under this Policy;
2. Any Pre-existing Disease or any complication arising therefrom.

Pre-existing Disease means any condition, ailment, injury or disease:

- a. That is/are diagnosed by a physician within 48 months prior to the effective date of the policy issued by the insurer or its reinstatement; or
 - b. For which medical advice or treatment was recommended by, or received from, a Physician within 48 months Prior to the effective date of the policy issued by the insurer or its reinstatement. Coverage under the policy after the expiry of 48 months for any pre-existing disease is subject to the same being declared at the time of application and accepted by Insurer.
3. Any Critical Illness caused due to treatment for, Alcoholism, drug or substance abuse or any addictive condition and consequences thereof.
 4. Narcotics used by the Life Assured unless taken as prescribed by a registered Medical Practitioner,

5. Any Critical Illness caused due to intentional self-injury, suicide or attempted suicide
6. Any Critical Illness caused by or arising from or attributable to a foreign invasion, act of foreign enemies, hostilities, warlike operations (whether war be declared or not or while performing duties in the armed forces of any country during war or at peace time), civil war, public defense, rebellion, revolution, insurrection, military or usurped power;
7. Any Critical Illness caused by ionizing radiation or contamination by radioactivity from any nuclear fuel (explosive or hazardous form) or from any nuclear waste from the combustion of nuclear fuel, nuclear, chemical or biological attack.
8. Congenital External Anomalies, or any complications or conditions arising therefrom including any developmental conditions of the Life Assured;
9. Any Critical Illness caused by any treatment necessitated due to participation as a professional in hazardous or adventure sport, including but not limited to, para jumping, rock climbing, mountaineering, rafting, motor racing, horse racing or scuba diving, hand gliding, sky diving, deep sea diving
10. Participation by the Life Assured in any flying activity, except as a bona fide, farepaying passenger of a recognized airline on regular routes and on a scheduled timetable.
11. Any Critical Illness caused by Medical treatment traceable to childbirth (including complicated deliveries and caesarean sections incurred during hospitalization) except ectopic pregnancy. Any Critical Illness caused due to miscarriages (unless due to an accident) and lawful medical termination of pregnancy during the policy period.
12. Any Critical Illness caused by any unproven/ experimental treatment, service and supplies for or in connection with any treatment. Unproven/ experimental treatments are treatments, procedures or supplies that lack significant medical documentation to support their effectiveness.
13. Any Critical Illness based on certification/diagnosis/treatment from persons not registered as Medical Practitioners, or from a Medical Practitioner who is practicing outside the discipline that he/ she is licensed for.
14. Any Critical Illness caused due to any treatment, including surgical management, to change characteristics of the body to those of opposite sex.
15. Any Critical Illness caused due to cosmetic or plastic surgery or any treatment to change the appearance unless for reconstruction following an Accident, Burn(s), or Cancer or as part of medically necessary treatment to remove a direct and immediate health risk to the Life Assured. For this to be considered a medical necessity, it must be certified by the attending Medical Practitioner.
16. Any Critical Illness caused due to surgical treatment of obesity that does not fulfil all the below conditions:

- a. Surgery to be conducted is upon the advice of the Doctor
- b. The Surgery / Procedure conducted should be supported by clinical protocols
- c. The Life Assured has to be 18 years of age or older and
- d. Body Mass Index (BMI):
 - i. greater than or equal to 40 or
 - ii. greater than or equal to 35 in conjunction with any of the following severe co-morbidities following failure of less invasive methods of weight loss:
 - i. Obesity related cardiomyopathy
 - ii. Coronary heart disease
 - iii. Severe Sleep Apnea
 - iv. Uncontrolled Type 2 Diabetes

17. Any Critical Illness caused due to treatments received in health hydros, nature cure clinics, spas or similar establishments or private beds registered as a nursing home attached to such establishments or where admission is arranged wholly or partly for domestic reason.

18. Any Critical Illness caused by treatment directly arising from or consequent upon any Life Assured committing or attempting to commit a breach of law with criminal intent.

19. In the event of the Death of the Life Assured within the stipulated survival period as set out above.

20. Any Critical Illness caused by sterility and infertility. This includes:

- a. Any type of contraception, sterilization
- b. Assisted Reproductive services including artificial insemination and advanced reproductive technologies such as IVF, ZIFT, GIFT, ICSI
- c. Gestational Surrogacy
- d. Reversal of sterilization

Statutory Information

Prohibition of rebate

The Insurance Act, 1938 prohibits an agent or any other person from passing any portion of his commission to the customer, whether as incentive or rebate of premium. Section 41 of the Act states:

- No person shall allow or offer to allow, either directly or indirectly, as an inducement to any person to take out or renew or continue an insurance in respect of any kind of risk relating to lives or property in India, any rebate of the whole or part of the commission payable or any rebate of the premium shown on the policy, nor shall any person taking out or renewing or continuing a policy accept any rebate, except such rebate as may be allowed in accordance with the published prospectuses or tables of the insurer.
- Any person making default in complying with the provisions of this section shall be liable for a penalty which may extend to ten lakh rupees.

Disclaimers

This brochure gives only the salient features of the Ageas Federal Life Insurance Criti Care Plan. It uses easy-to-understand language to explain the features. Your plan is governed only by the full legal terms, conditions and exclusions as contained in the policy document. Please read the policy document for more details.

Ageas Federal Life Criti Care Plan (UIN: 135N085V01) is a Non-linked, Non-participating, Individual, Health, Insurance Plan and no benefits other than those indicated in this brochure are payable. There are no riders attached to this plan. This product does not participate in the profits of the Company. Substandard lives will be charged extra premium. This product is underwritten by Ageas Federal Life Insurance Company Limited (*Formerly known as IDBI Federal Life Insurance Company Limited*), (Regn. No 135; Corporate Identity Number (CIN) – U66010MH2007PLC167164) having its registered office at 22nd Floor, A Wing, Marathon Futurex, N. M. Joshi Marg, Lower Parel – East, Mumbai – 400013.

Tax Benefits are as per the Income Tax Act, 1961 and are subject to changes in the tax from time to time. Ageas Federal Life Insurance Company Limited does not assume responsibility on tax implication. Trade Logo displayed above belongs to The Federal Bank Limited and Ageas International Insurance N. V. and used by Ageas Federal Life Insurance Company Limited under license from respective partners. Ageas Federal Life Insurance Company Ltd. does not assume responsibility on tax implication. ARN: 19094/CICP/ENG/Print-PB/Jun24

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IRDAI clarifies to public that

IRDAI is not involved in activities like selling insurance policies, announcing bonus or investment of premiums. Public receiving such phone calls are requested to lodge a police complaint.

Contact Us



Branches

Visit or call any branch of Ageas Federal Life Insurance Co Ltd., Federal Bank or IDBI Bank. For the list of branches, please visit www.ageasfederal.com



Phone

Call our nationwide toll free number **1800 209 0502** from Monday to Saturday at any time between 8 am to 8 pm.



Website

Visit our website www.ageasfederal.com



Write

Write to customer service desk:

Ageas Federal Life Insurance Company Limited
(Formerly known as IDBI Federal Life Insurance Co. Ltd.),
22nd Floor, A Wing, Marathon Futurex, N. M. Joshi Marg,
Lower Parel – East, Mumbai – 400013



Email

Email us at:

support@ageasfederal.com